

MIST. CARMINATIVE (ADULT)

DESCRIPTION

A strong cardamon flavoured, buff coloured suspension.

Each 5ml contains:

150mg of Light Magnesium Carbonate

250mg of Sodium Bicarbonate

This product contains 0.005% Ethyl Alcohol.

PHARMACODYNAMIC

Light magnesium carbonate and sodium bicarbonate act as an antacid by neutralising the acid secretion in the stomach.

PHARMACOKINETIC

Magnesium Carbonate:

It interacts with gastric acid in the stomach to form soluble magnesium chloride and carbon dioxide. Magnesium carbonate is absorbed but usually excreted rapidly in the urine.

Sodium Bicarbonate:

Oral bicarbonate, such as sodium bicarbonate, neutralizes gastric acid with the production of carbon dioxide. Bicarbonate not involved in that reaction is absorbed, and in the absence or deficit of bicarbonate in the plasma, bicarbonate ions are excreted in the urine.

INDICATIONS

Used as an antacid in gastric disorders to relief symptoms of gastric hyperacidity, variously called indigestion, heartburn, dyspepsia and flatulence.

RECOMMENDED DOSAGE

10 to 20 ml, 3 – 4 times daily or as prescribed by physician.

Shake the bottle before use.

Antacids are normally given between meals and at bedtime when symptoms of gastric hyperacidity usually occur; the presence of food in the stomach can prolong the neutralizing activity.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

Hypersensitivity to any of the ingredients, renal impairment, hypophosphatemia and patients on a sodium restricted diet (in heart failure, hepatic and renal impairment, liver cirrhosis and during pregnancy), hypocalcemia, hypochlorhydria, metabolic and respiratory alkalosis because of the sodium bicarbonate content.

WARNINGS & PRECAUTIONS

If symptoms persist, consult your doctor. Caution should be exercised in elderly or debilitated patients, patients with hypertension, oedema, eclampsia, aldosteronism, respiratory acidosis and patients receiving corticosteroids. Sodium

bicarbonate compound should not generally be given to patients with hypochlorhydria.

INTERACTIONS WITH OTHER MEDICAMENTS

Antacids may interact with a number of other drugs by altering their absorption and, sometimes, their elimination, thereby reducing their effectiveness. Antacids may also damage enteric coating designed to prevent dissolution in the stomach. In order to minimise the risk of interactions, this product should not be taken within two to four hours of other medications (allow at least 4 hours before or 2 hours after erlotinib).

Examples of other medications which may be affected include, but are not limited to ACE inhibitors, salicylates e.g. aspirin, atazanavir, azithromycin, barbiturates, bile acids, bisphosphonates, cephalosporin antibiotics, fluoroquinolone antibiotics, chloroquine and hydroxychloroquine, deflazacort, digoxin, dipyridamole, eltrombopag, erlotinib, fexofenadine, gabapentin, iron preparations, isoniazid, itraconazole, ketoconazole, lansoprazole, levothyroxine, lithium, methenamine, mycophenolate, nitrofurantoin, penicillamine, phenothiazines, phenytoin, proguanil, rifampicin, rosuvastatin, sulpiride, tetracyclines, tipranavir, ulipristal (avoid use with antacids).

There is a risk of metabolic alkalosis when oral magnesium salts are given with polystyrene sulphonate resins.

PREGNANCY & LACTATION

Pregnancy:

Caution should be exercised when prescribing to pregnant women as this medicine contains sodium. Animal studies are insufficient with respect to effects on pregnancy, embryonal/fetal development, parturition and postnatal development. This medicine should not be taken during the first three months of pregnancy.

Lactation:

This medicine should not be used during lactation unless clearly necessary.

ADVERSE EFFECTS

Magnesium salts may cause diarrhoea in some patients. Magnesium carbonate and sodium bicarbonate may cause stomach cramps and flatulence as a result of excess carbon dioxide production.

Long-term, excessive use has been associated with the development of silica-based renal calculi.

Rebound hyperacidity may occur with prolonged dosage.

SYMPTOMS AND TREATMENT FOR OVERDOSAGE

Symptoms:

Overdose, or excessive or prolonged intake of magnesium containing antacids may give rise to hypermagnesemia, and excessive administration of sodium bicarbonate may lead to hypokalemia and metabolic alkalosis, especially in patients with renal insufficiency.

Symptoms of hypermagnesemia include nausea, vomiting, flushing of the skin, thirst, drowsiness, hypotension, confusion, muscle weakness, central nervous system (CNS) and respiratory depression, hyporeflexia, peripheral vasodilatation, bradycardia, cardiac arrhythmias, coma and cardiac arrest.

Symptoms of hypokalemia and metabolic alkalosis include mood changes, tiredness, shortness of breath, muscle weakness and irregular heartbeat. Muscle hypertonicity, twitching and tetany may develop, especially in hypocalcemia patients. Excessive doses of sodium salts may lead to sodium overloading and hyperosmolality.

Treatment:

Treatment of mild hypermagnesemia is usually limited to restricting magnesium intake. In severe hypermagnesemia, ventilatory and circulatory support may be required. Treatment should consist of the intravenous administration of calcium gluconate injection 10% at a dose of 10 – 20ml, to counteract respiratory depression or heart block. If renal function is normal, adequate fluids should be given to assist magnesium removal from the body.

Hemodialysis may be necessary in patients with renal impairment or for whom other methods prove ineffective. Metabolic alkalosis and hypernatremia can be treated by appropriate correction of fluid and electrolyte balance. Replacement of calcium, chloride, and potassium ions may be of particular importance.

PACK SIZE

Plastic Container of 120ml.

STORAGE CONDITIONS

Store below 30°C, away from direct light.

INSTRUCTIONS FOR USE

Not applicable

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

REGISTRATION NUMBER

MAL19912224X

**KEEP MEDICINES OUT OF REACH OF CHILDREN
JAUHI UBAT-UBATAN DARI KANAK-KANAK**

For further information, please consult your doctor or your pharmacist.

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Lt-178.04 Product Registration Holder and Manufacturer
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